

Name: مديحة زكي ابو خزيمة موسى
National ID: 27504162201085



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :

PM 01:17 05-07-2026

تاريخ وساعة الدخول :

---:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي (Phesgo (Subcutaneous trastuzumab and pertuzumab

الوقت :

---:--

التاريخ :

05-07-2026

الطبيب المعالج :

د/شيماء لاثنين

Patient Name: MADIHA ZAKY ABO KHOZIEM	Patient ID: MADIHA ZAKY ABO KHOZIEM
Procedure:	Age:
Study Date: 18/04/2026 06:27:19 PM	Report Date: 18/04/2026 06:36:50 PM

ULTRASONOGRAPHIC SCANNING EXAMINATION OF THE ABDOMEN AND PELVIS REVEALED:

Liver:

- Is seen of average size, it shows smooth surface and sharp edges. Parenchyma presents uniform pattern with normal internal structures in both lobes. No defects or discrete masses are noted; no dilated bile ducts are seen.

Gall bladder: is seen harboring solitary stone about 8 mm. in diameter, the gall bladder wall is not seen thickened and there is no biliary mud.

Spleen: is not enlarged, pattern is homogenous.

Pancreas: appears concealed with gases.

Both kidneys:

- Both kidneys are of normal size, shape and pattern. No evident obstructive or back pressure changes. No renal stones or masses are seen.

There is no evidence of free ascites.

Echo free urinary bladder with no masses or stones.

Anteverted uterus with no masses identified.

Right ovarian unilocular thin wall cyst is seen it measures about 8.6x6.7 cm. in diameters.

No pelvic masses or collections.

Conclusion:

- Gall bladder stone.
- Right ovarian cyst.

Dr. Mohamed Salah, MD

Name: <u>مديحه ركي ابو حريم موسى</u>		
MR. : <u>10437</u>		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 51-year-old premenopausal individual with a history of hypertension and osteoporosis who was diagnosed with invasive ductal carcinoma (IDC) of the left breast, grade 3, estrogen receptor 0/8, progesterone receptor 0/8, HER2 + 3. Contrast-enhanced spectral mammography demonstrated left breast outer-central multifocal enhancing masses measuring 3.7 cm and 1.9 cm, interconnected over a 6.7 cm span with intraductal extension to the nipple, and suspicious intramammary, level III axillary, and supraclavicular lymph nodes. Staging computed tomography of the chest, abdomen, and pelvis was otherwise unremarkable except for a right renal cyst measuring 8.5 × 7 × 6.5 cm, for which a Portacath insertion was planned. The patient was scheduled to begin neoadjuvant chemotherapy (NACT) with four cycles of AC followed by four cycles of TCHP. Baseline echocardiogram showed an ejection fraction of 62 %. Laboratory results were pending at the time of planning. During cycle 2 of AC, the patient presented with mild unilateral lower-leg edema, prompting a venous duplex study. Laboratory results remained pending at that visit. The patient was seen in the daycare pharmacy for management of elevated liver enzymes, and liver support measures were initiated. Laboratory studies obtained prior to the third cycle of AC demonstrated improvement in liver enzyme levels, and the patient reported no new complaints. The patient has now completed three cycles of AC.		
تحريراً في ,		
	التاريخ :	19-09-2026
	الوقت :	شريف الشاوي
الطبيب المعالج :		
يعتمد ,		

- Findings are highly suspicious (BI-RADS IVc)
- Histopathological assessment is strongly recommended

Dr. Ahmed Sayed (M.D)
Radiology consultant

دكتور
احمد سيد ابراهيم
مستشار الامانة العامة

**NADIA
MOKHTAR**

Professor of Pathology
Cancer Institute, Cairo University

دكتورة
نادية محمود مختار
أستاذ الباثولوجي
معهد الأورام - جامعة القاهرة

Patient Name : مديحه زكى ابو حزين

Age : 51 Y

Referred by Prof. Dr : أ.د/منى كمال

Clinical Diagnosis : LEFT BREAST MASS

Nature of the Specimen : TRU-CUT BIOPSY

Report Date : 12 / 4 / 2026

Pathology Number : 25248



PATHOLOGY REPORT

GROSS:

Multiple (5) grayish-white tissue cores, the longest measured 2 cm, were totally submitted

MICROSCOPIC:

Sections examined revealed breast tissue cores infiltrated by malignant growth, displayed as clusters, nests and tubules of high anaplastic malignant ductal cells surrounded by desmoplastic stroma. No remarkable TILs. No DCIS, lymphovascular emboli or perineural invasion seen.

ER, PR, HER-2, KI-67 could be done.

DIAGNOSIS:

LEFT BREAST MASS, TRU-CUT BIOPSY; INVASIVE DUCT CARCINOMA GIII.

Prof Dr. Nadia Mokhtar
Professor of Pathology , PhD, FRCPath

ك / عصمت محمد احمد حسين

دبلومة الكيمياء الحيوية

كلية علوم - جامعة القاهرة



عمل المدينة المنورة

للتحليل الطبية الكيميائية

Pat No	:	12181176-0	Reg Date	:	29-03-2026 05:27
Lab Code	:	12199564	Sex	:	Female
Name	:	مديحه زكي	age	:	51 Years
Referred by	:	المدينه المنوره ابو صبير	Reporting Date	:	29/03/2026 07:13

FERTILITY REPORT

Test	Result	Unit	Ref. Range
Prolactin	3.0	ng/ml	2.9 - 29.9

Pathology Lab
معمل الباثولوجي
Pathology request form

*Patient Name:

صديقه زكي الوضحي

*Date of Birth:

Female

*Hospital Number:

Male

*Sex:

*Requesting Physician:

د. الزاهد

*Department:

جراحة

*Clinical data:

breast d

Specimen data:

Number Of specimens:

	*Time Taken	*Time into Fixative
1. <i>Stoek version</i>	:	:
2.	:	:
3.	:	:
4.	:	:
5.	:	:

Department/Site

Number of Containers
()

Sent by

Lab Use Only

Demographics Checked	Initials
Received number of Containers ()	
Accessioning	
Grossing	
Embedding/Microtomy:	
Pathologist	

Time out of Formalin

:

Time submitted to pathologist

:

*Must be filled.

Form code: F/SOP/PL/20/01

Issue date: 9/3/2021

Version No.: 2/0



ECHOCARDIOGRAPHY

REPORT

تقرير الموجات الصوتية علي القلب

اسم المريض: : مديحة زكي ابو خزين

Patient name:	Madiha Zaki Abo Khozain	Age:	51	Years
Date:	18/04/2026	Sex:	Female	

Measurements

		NORMAL			NORMAL
LVEDD	4.4	2.3-5.7CM	LA	4.1	2.5-4CM
LVESD	3.0		AO	3.1	2.8-3.6CM
SWT	1.1	0.5-1.1CM	RV		1.6-2.6CM
PWT	1.0	0.5-1.1CM			
FS					
EF	62%				

Comment:

- ⊗ Normal LV end systolic dimensions with normal wall thickness and normal global and regional systolic function.
- ⊗ No SWMA at rest.
- ⊗ Dilated left atrium.
- ⊗ Normal MV thickness and excursion with no mitral regurgitation or significant stenosis.
- ⊗ Mitral inflow shows pseudo-normal E/A ratio.
- ⊗ Aortic valve trileaflet with adequate leaflets thickness without significant stenosis or regurgitation.
- ⊗ Normal RV dimensions with preserved systolic function.
- ⊗ Intact inter-atrial and inter-ventricular septum
- ⊗ Normal appearance of the pericardium. No effusion
- ⊗ No intra-cardiac masses, thrombi or vegetation seen by TTE

Conclusions:

- Normal LV end systolic dimensions with normal global and regional systolic
- Dilated left atrium

Dr. Khaled Alaa, Msc

Patient Name السيدة/ مديحة تكي ابو خزيم
Age / Sex 51 Years / Female
Referred By Herself

Patient ID. 
Request Date 15-Apr-2026 08:17 PM
Printed In 16-Apr-2026 01:32 PM

BIOCHEMISTRY REPORT

Kidney Functions :

1 Serum Creatinine : 0.86 mg/dL

Reference Range :

Normal: 0.6 - 1.1 mg/dL

Doctor's signature:
Enas Mohamed Abdelghafar
Thanks



مركز تنمية كفاءات الكوادر التعليمية
مملكة البحرين

ملاحظة

زكي ابوكرزيم موسى

ابو صبر الملقب

مركز الواسطي - بنى سويف

١٩٧٥/٠٤/٢٥

٧٧٥٠٤١٦٢٢٠١٠٨٥

LM0851322



٢٠٢٥/١٢
٧٧٥٠٤١٦٢٢٠١٠٨٥
ربة منزل

ملاحظة

يوسف عبدالحميد جعفر زيان

البلقاء سارية حتى ٢٠٢٢/١١/٠٧



Patient Name	مريضة زكي ابو خريم	Case No.	201279
Sex/Age	Female 51 Y	Patient No.	17759
Registered Date	13/5/2026 10:28:49 AM	Referred By Dr.	Out Patient
Validation Date	02/6/2026 01:23:09 PM	Branch	Gustave Roussy
Reporting Date	18/7/2026 12:56:02 PM	Pathology No.	SR-577/26

Clinical Data: Left breast mass.

Breast Slide Revision

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (25248). Together with, five referred slides including 1 (H&E) stained slide and 4 immunostained slides for (ER, PR, Her2, Ki67).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
 - * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
 - * Ductal carcinoma in situ: Absent.
 - * Angioinvasion: Absent.
 - * Perineural invasion: Absent.
 - * Microcalcification: Present.
- IPT:**
- ER: Negative (score 0/8).
 - PR: Negative (score 0/8).
 - Her2: Positive (score 3+).
 - Ki67%: 20%.

Diagnosis:

Left breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 3.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67%: 20%.

* *Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%



مركز باثولوجيا روستو الدولي - مستشفى روستو الدولي - (1987) - 141 شارع روستو - القاهرة - مصر
 Report No. (4) - First Floor-Building No. 987 - Nile Corniche - Nasser El Ghattema - Cairo, Egypt

Patient Name	مريضة زكي ابو خزيمة	Case No.	201279
Sex/Age	Female 51 Y	Patient No.	17759
Registered Date	13/5/2026 10:28:49 AM	Referred By Dr.	Out Patient
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and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

*** Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within 10% of the invasive tumor cells. IHC 3+, based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim



Room No. (4) - First Floor-Building No. 987, Nile Courtyard, Alve El Qadima - Cairo, Egypt.

Patient's Name:

Madiba Zaki Abu Khuzaim Musa

Date:

2026-05-10

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

(I) Chest:

- left breast ill defined mass LOQ measuring 19 x 11mm with architectural distortion
- multiple enlarged left axillary lymph nodes , the largest measures 14 x 16 mm
- Patent central air ways with well inflated both lungs.
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is average in size showing homogenous CT density with no enhancing focal lesions
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.

Patient's Name:

Madiha Zaki Abu Khuzaim Musa

Date:

2026-05-10

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Large right sided cyst 8.5 x 7 x 6.5 cm along its maximum dimension , no enhancing solid componentlikely related to right adnexa
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

- **(III) Examined skeleton:**

- No worrisome lytic or sclerotic bony lesions
- No vertebral structural collapse or wedging.

OPINION:

No pulmonary , hepatic or bony metastatic deposits.

Right adnexal cyst , no solid component .

Dr.Reham Khedr, MD

Dr.Rahma Khader

Dr.Rahma Khader

Dr.Rahma Khader

Patient's Name:

Madha

Zaki Abu Khuzaim Musa



Date:

2026-05-21

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: left palpable mass

Findings:

Right breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated multiple scattered small simple cysts
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogenously dense breast parenchyma that may obscure small lesions. (ACR c)
- Ultrasound demonstrated scattered simple cysts.
- 4 o'clock, irregular spiculated hypoechoic solid mass lesion with internal vascularity on color measuring 30 x 20 mm, 14 mm from the skin , and 5 cm from the nipple
- Normal skin/nipple/areola complex.
- Multiple suspicious axillary LNs are noted showing eccentric hilum the largest measuring 18 x 17 mm with cortical thickening of 9 mm
- free supra and infraclavicular regions

Conclusion:-

- **Right breast (BIRADS 2): fibrocystic changes**
- **Left breast (BIRADS 5): 4 o'clock suspicious mass lesion with ipsilateral suspicious axillary lymph nodes for histopathological correlation**

Dr farah elshobaky

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MDT FORM			
Name:	اسم المريض	MRN:	10437
Age:	51 Y	ID:	
Menopausal Sta		Phone:	01280952651
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	HTN	Reason Of MDT:	new case

Clinical Data:

R

BX: IDC G3
 ER: C1B PR: O1B HER2: +3 KI67: 20%

Sonomammography:

CESN
 LT OUTER, CENTRAL MIDDLE AND POSTERIOR THIRD 2 INTERCONNECTED DENSE IRREGULAR SPECULATED MASSES, HETEROGENOUS ENHANCING MASSES 3.7*3.1*3.4 CM & 1.9*1.7*0.7 CM, OVERALL AREA 6.7*3.1*3.4 CM 4-5 ZONE B&C WITH INTRADUCTAL EXTENSION REACHING NIPPLE
 3 ZONE C BITHANIMARY LN NON UNIFORM CORTICAL THICKNESS UP TO 0.5CM, 1.3*1 CM
 LEVEL 3 DISCRETE PATHOLOGICAL LN, THICKNESS 0.9 CM MEASURE 1.5*1 CM

CH
 MULTIPLE LT SUPRACLAVICULAR NOOES 0.8*0.7 CM WITH CORTICAL THICKNESS 0.5CM

Mets Workup:

CT CAP: FREE EXCEPT
 RT CYST 8.5*7*6.5 CM

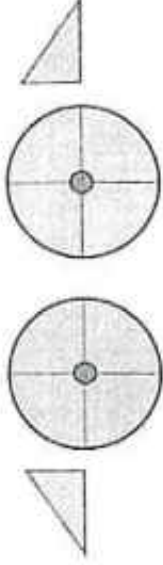
History:

Recommendations:
 FOR NACT THEN LT MRM

MDT Outcome & Plan

Being N3
 Portacath insertion
 For NACT 4 AC - 4 TCHP
 THEN LT MRM

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هبة عبد الحليم	د. هبة عبد الحليم				

Patient Name	مريضة زكى ابو خريم 0	Code Number	YK 201279
Sex/Age	Female 51 Y	Patient ID	17759
Registered Date	13/5/2026 10:28:49 AM	Referred By Dr.	Out Patient
Validation Date	02/6/2026 01:23:09 PM	Branch	Gustave Roussy
Reporting Date	02/6/2026 01:31:51 PM	Pathology No.	SR-577/26

Breast Slide Revision

Clinical Data: Left breast mass.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

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Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
 - * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
 - * Ductal carcinoma in situ: Absent
 - * Angioinvasion: Absent
 - * Perineural invasion: Absent
 - * Microcalcification: Present
- IPT:**
- ER: Negative (score 0/8).
 - PR: Negative (score 0/8).
 - Her2: Positive (score 3+).
 - Ki67%: 20%.

Diagnosis:

Left breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 3.
- IPT:**
- ER: Negative (score 0/8).
 - PR: Negative (score 0/8).
 - Her2: Positive (score 3+).
 - Ki67%: 20%.

Patient Name	ملیحه زکی ابو خزیم 0	Code Number	YK 201279
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* * Scoring system: "ER, PR"

*Scoring system: "ER, PR" ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%; 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, moderate; 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

*****Scoring system: "Her2"**
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IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within <10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense >10% of tumor cells.

Signature

Dr. Walad Ibrahim Ghoneim



كود المدخل	كود المراجع	كود الطبيب
٢٨٦٦٤	٨٢٤٠	٢١٨٢٢

توصية المجلس الطبي المتخصص

رقم ٣٩٤٢١٣١٤ لسنة ٢٠٢٦
الرقم القومي ٢٧٥٠٤١٦٢٢٠١٠٨٥

الإدارة العامة للمجالس الطبية المتخصصة

- بعد الاطلاع علي تقرير الكشف الطبي والابحاث الخاصة بالمواطن المذكور ادناه
- ووفقا لما جاء بقرار وزير الصحة رقم ٤٠٨ لسنة ٢٠٠٩
- وعلي تقرير اللجنة الثلاثية الصادر من المستشفى بتاريخ ٢٠٢٦/٠٥/٠١
- و المسجل علي النظام بتاريخ ٢٠٢٦/٠٥/٠١
- اوصي المجلس الطبي المتخصص بجلسة ٢٠٢٦/٠٥/٠٤

بالموافقة علي علاج المواطن / مديحة زكي ابو خريم موسى من سرطان بالثدي (مبادرة) بمستشفى جوستاف روسي انترناشيونال - مصر (EPE) * بقسم الاورام خارجي مامو جرام وسونار علي الثديين (كود ١٠٨٨٨) .
وفقا لما جاء بأسعار بروتوكولات العلاج علي نفقة الدولة وطبقا لكود الحالة المرضية

تحرير ا في ٢٠٢٦/٠٥/٠٤

تعتبر التوصية نافذه من تاريخ تحرير اللجنة الثلاثيه

لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض ومناظرة المريض بالمستشفى قبل الصرف
قيمة القرار مخصصة للعلاج ولا يجوز المطالبة بأي نفقات أخرى سواء للإنتقال أو أي إجراء خارج منطوق القرار

مدير عام

مراجعة طبية

مراجعة ادارية

الإدارة العامة للمجالس الطبية المتخصصة

د/ وسلم محمود عبدالعزیز -

سالم صموئيل تالوروس

د/ محمد عفتان

جوستاف روسي انترناشيونال - مصر (EPE) *
طبع في: ٢٠٢٦/٠٥/١٠ ٢٥٨٢٧٣٤٢



وحدة دلالات الأورام
قسم الباثولوجيا - قصر العيني
ت : ٠١٠٦٨٥٢٩٦٨٤

IMMUNOHISTOPATHOLOGY

Patient Name: مديحة زكي أبو حزين

lab.No:2026-585

Date:3/5/ 2026

Age: 51Y

Sex: Female

Markers: ER, PR, Her 2 neu and Ki67 .

Procedure:

Sections were prepared from paraffin blocks treated for (ER-clone SP1), (PR clone 1E2), (Her 2 neu-clone 4B5) and (Ki67 - clone 30-9) automatically stained in Bench mark ULTRA (VENTANA) using VIEW DAB Detection Kit, Hematoxylin as counter stain

Results:

Examination of sections treated for ER revealed no nuclear staining of tumor cells by chromogen.

Examination of sections treated for PR revealed weak nuclear staining <10%of tumor cells by chromogen

Examination of sections treated for Her 2 neu revealed strong complete membranous staining in >10% of tumor cells by chromogen

Examination of sections treated for Ki67 revealed positive nuclear staining in 30 % of tumor cells by chromogen.

DIAGNOSIS: Left breast mass, tru cut biopsy; PT:

- NEGATIVE FOR ER SCORE (0/8)
- POSITIVE FOR PR SCORE (3/8)
- POSITIVE FOR HER2 (SCORE3+)
- Ki67: 30%

PROF. DR. SAMIA GABAL
PROF. DR. MONA SALAH

د. سامية غبال
د. منة صلاح

كود المتخذ	كود المراجعة	كود الطبيب
٢٨٦٦٤	٨٢٤٠	٢١٨٢٢

توصية المجلس الطبي المتخصص

رقم ٣٩٤٢٥٢٧٠ لسنة ٢٠٢٦
الرقم القومي ٢٧٥٠٤١٦٢٢٠١٠٨٥

الادارة العامة للمجالس الطبية المتخصصة

- بعد الاطلاع علي تقرير الكشف الطبي والابحاث الخاصة بالمواطن المذكور ادناه
- ووفقا لما جاء بقرار وزير الصحة رقم ٤٠٨ لسنة ٢٠٠٩
- وعلي توصية المجلس الطبي المتخصص رقم ٣٩٤٢١٣١٤ لسنة ٢٠٢٦/٠٥/٠٤
- وعلي تقرير اللجنة الثلاثية الصادر من المستشفى بتاريخ ٢٠٢٦/٠٥/٠١
- والمسجل علي النظام بتاريخ ٢٠٢٦/٠٥/٠١
- اوصي المجلس الطبي المتخصص بجلسة ٢٠٢٦/٠٥/٠٥

بالموافقة علي استكمال علاج المواطن / مديحة زكي ابو خزيم موسى من سرطان بالثدي (مبادرة) الصادر في شأنه التوصية رقم ٣٩٤٢١٣١٤ بمستشفى جوستاف روسي انترناشيونال - مصر (EPE) * بقسم الاورام خارجى اشعه مقطعيه علي الصدر والبطن والحوض بالصبيغه (كود ١١١٢٩) .

وفقا لما جاء باسعار بروتوكولات العلاج علي نفقة الدولة وطبقا لكود الحالة المرضية

تحرير ا في ٢٠٢٦/٠٥/٠٥

تعتبر التوصية نافذة من تاريخ تحرير اللجنة الثلاثية

لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض ومناظرة المريض بالمستشفى قبل الصرف قيمة القرار مخصصة للعلاج ولا يجوز المطالبة بأي نفقات أخرى سواء للإنتقال أو أي إجراء خارج منطوق القرار

مدير عام

مراجعة طبية

مراجعة ادارية

الإدارة العامة للمجالس الطبية المتخصصة

د/وسلم محمود عبدالعزیز -

ساح صموئيل تاوروس

د/ محمد عقار

جوستاف روسي انترناشيونال - مصر (EPE) *
طبع في : ٢٠٢٦/٠٥/١٠ ٦٥٨٧٢٣٠

وزارة الصحة
رئيس الجمهورية



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

وزارة الصحة
رئيس الجمهورية

met/dd/yyyy

MDT FORM

Name:	اسم المريض	MRN:	10437
Age:	51 Y	ID:	
Menopausal Sta		Phone:	01280952651
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	HTN	Reason of MDT:	new case

Clinical Data:

BX:	IDC G3	PR:	0/8	HER2:	+3	KI67:	20%
ER:	0/8	PR:	0/8	HER2:	+3	KI67:	20%

R

L

Sonomammography:

CEM

LT OUTER CENTRAL MIDDLE AND POSTERIOR THIRD 2 INTERCONNECTED DENSE IRREGULAR SPECULATED MASSES , HETEROGENEOUS ENHANCING MASSES 3.7*3.1*3.4 CM & 1.9*1.7*0.7 CM , OVERALL AREA 6.7*3.1*3.4 CM 4:5 ZONE BAC WITH INTRADUCTAL EXTENSION REACHING NIPPLE 3 ZONE C INTRAMAMMARY LN NON UNIFORM CORTICAL THICKNESS UP TO 0.5CM , 1.3*1 CM LEVEL 3 DISCRETE PATHOLOGICAL LN , THICKNESS 0.9 CM MEASURE 1.5*1 CM

MULTIPLE LT SUPRACLAVICULAR NODES 0.8*0.7 CM WITH CORTICAL THICKNESS 0.5CM

Mets Workup:

CT CAP : FREE EXCEPT
RT CYST 8.5*7*6.5 CM

History:

Recommendations:

FOR NACT THEN LT MRM

MDT Outcome & Plan

Being N3
Portacath Insertion
For NACT 4 AC - 4 TCHP
THEN LT MRM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

T N M



Patient Name: MADIHA ZAKY ABO KHOZIEM	Patient ID: MADIHA ZAKY ABO KHOZIEM
Procedure:	Age:
Study Date: 18/04/2026 06:27:19 PM	Report Date: 18/04/2026 06:36:50 PM

ULTRASONOGRAPHIC SCANNING EXAMINATION OF THE ABDOMEN AND PELVIS REVEALED:

Liver:

- Is seen of average size, it shows smooth surface and sharp edges. Parenchyma presents uniform pattern with normal internal structures in both lobes. No defects or discrete masses are noted; no dilated bile ducts are seen.

Gall bladder: is seen harboring solitary stone about 8 mm. in diameter, the gall bladder wall is not seen thickened and there is no biliary mud.

Spleen: is not enlarged, pattern is homogenous.

Pancreas: appears concealed with gases.

Both kidneys:

- Both kidneys are of normal size, shape and pattern. No evident obstructive or back pressure changes. No renal stones or masses are seen.

There is no evidence of free ascites.

Echo free urinary bladder with no masses or stones.

Anteverted uterus with no masses identified.

Right ovarian unilocular thin wall cyst is seen it measures about 8.6x6.7 cm. in diameters.

No pelvic masses or collections.

Conclusion:

- Gall bladder stone.
- Right ovarian cyst.

Dr. Mohamed Salah, MD

TIBA SCAN CENTER

Name: Many Thatt And Excellence



طبية الأشعة

Name: Mostafa Zaki Ahmed Hossain Moustafa
Date: 25/04/2026

Ref: Prof. Dr. Mona Kamal Othman

Left Breast Mammography with Complementary Ultrasonography

- Technique:**
 - Digital mammography of the left breast performed in standard cranio-caudal (CC) and medio-lateral oblique (MLO) views
 - Targeted ultrasound of the left breast and axilla performed using a high-frequency linear transducer
- Breast Parenchyma (Mammography):**
 - Heterogeneously increased breast density is noted
- Ultrasound Findings (Left Breast):**
 - A rather well-defined hypoechoic mass lesion is identified at the lower outer quadrant corresponding to 4 o'clock position
 - The lesion measures approximately 22 x 26 mm
 - The lesion demonstrates an irregular outline
 - Mild internal vascularity is noted on color Doppler interrogation
 - Average caliber of lactiferous ducts
- Left Axilla:**
 - Few enlarged lymph nodes are identified
 - Lymph nodes are oval in shape with preserved echogenic fatty hilum
 - Mild cortical thickening is noted
 - The largest lymph node measures approximately 12 x 21 mm
- Impression:**
 - Irregular hypoechoic mass lesion in the left breast at 4 o'clock measuring approximately 22 x 26 mm with internal vascularity
 - Associated left axillary lymph nodes with mild cortical thickening
 - Findings are highly suspicious (BI-RADS IVc)
 - Histopathological assessment is strongly recommended

دكتور
احمد سيد ابراهيم
مستشار الاشعة التليفزيونية

Best regards,
Dr. Ahmed Sayed (M.D.)
Radiology consultant

المركز الرئيسي - بني سويف، شارع بورسعيد - برج الندى - أمام معرض شباب مصر
☎ 01100774888 - 01200424450 - 01060726687 - 0822350159 - 0822350153

فرع 1 - بني سويف - شارع بورسعيد - برج الحياة - أمام مرافق المديرية ☎ 01163598180

فرع 2 - بني سويف - من أحمد عزاني - برج سيناء الطبي - بجوار مستشفى الزهراء ☎ 01124876922

**NADIA
MOKHTAR**

Professor of Pathology
Cancer Institute, Cairo University

دكتورة
نادية محمود مختار
أستاذ الباثولوجي
معهد الأورام - جامعة القاهرة

Patient Name : مديحه زكى ابو حزين

Age : 51 Y

Referred by Prof. Dr : أ.د/منى كمال

Clinical Diagnosis : LEFT BREAST MASS

Nature of the Specimen : TRU-CUT BIOPSY

Report Date : 12 / 4 / 2026

Pathology Number : 25248



PATHOLOGY REPORT

GROSS:

Multiple (5) grayish-white tissue cores, the longest measured 2 cm, were totally submitted

MICROSCOPIC:

Sections examined revealed breast tissue cores infiltrated by malignant growth, displayed as clusters, nests and tubules of high anaplastic malignant ductal cells surrounded by desmoplastic stroma. No remarkable TILs. No DCIS, lymphovascular emboli or perineural invasion seen.

ER, PR, HER-2, KI-67 could be done.

DIAGNOSIS:

LEFT BREAST MASS, TRU-CUT BIOPSY; INVASIVE DUCT CARCINOMA GIII.

Prof Dr. Nadia Mokhtar
Professor of Pathology , PhD, FRCPath



جمهورية مصر العربية
بطاقة تحقيق الشخصية

مديحه

زكى ابو خريم موسى

ابوصير الملق

مركز الواسطى - بنى سويف

١٩٧٥/٠٥/١٦

٢٧٥٠٤١٦٢٢٠١٠٨٥

K00911229

ك / عصمت محمد احمد حسين
دبلومة الكيمياء الحيوية
كلية علوم - جامعة القاهرة



عمل المدينة المنورة
للتحليل الطبية الكيميائية

Pat No	:	12181176-0	Reg Date	:	29-03-2026 05:27
Lab Code	:	12199564	Sex	:	Female
Name	:	مديحه زكي	age	:	51 Years
Referred by	:	المدينه المنوره ابو صبير	Reporting Date	:	29/03/2026 07:13

FERTILITY REPORT

Test	Result	Unit	Ref. Range
Prolactin	3.0	ng/ml	2.9 - 29.9



٢٠٢٤/٠٥

٢٧٥.٤١٦٢٢.١.٨٥

ربة منزل



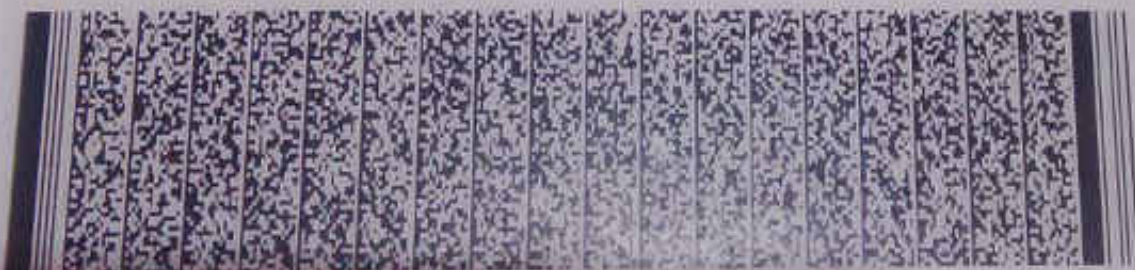
متزوجة

مسلمة

انثى

يسن عبدالحميد جمعه زيدان

البطاقة سارية حتى ٢٠٣١/٠٥/٠٧



شؤون

١٦٠

١٥

١٨

Pathology Lab
معمل الباثولوجي
Pathology request form

*Patient Name:

صديقه زكي الوضحي

*Date of Birth:

Female

*Hospital Number:

Male

*Sex:

*Requesting Physician:

د. الزاهد

*Department:

جراحة

*Clinical data:

breast d

Specimen data:

Number Of specimens:

	*Time Taken	*Time into Fixative
1. <i>Sholek version</i>	:	:
2.	:	:
3.	:	:
4.	:	:
5.	:	:

Department/Site

Number of Containers
()

Sent by

Lab Use Only

Demographics Checked	Initials
Received number of Containers ()	
Accessioning	
Grossing	
Embedding/Microtomy:	
Pathologist	

Time out of Formalin

:

Time submitted to pathologist

:

*Must be filled.

Form code: F/SOP/PL/20/01

Version No.: 2/0

Issue date: 9/3/2021



ECHOCARDIOGRAPHY

REPORT

تقرير الموجات الصوتية علي القلب

اسم المريض: : مديحة زكي ابو خزين

Patient name:	Madiha Zaki Abo Khozain	Age:	51	Years
Date:	18/04/2026	Sex:	Female	

Measurements

		NORMAL			NORMAL
LVEDD	4.4	2.3-5.7CM	LA	4.1	2.5-4CM
LVESD	3.0		AO	3.1	2.8-3.6CM
SWT	1.1	0.5-1.1CM	RV		1.6-2.6CM
PWT	1.0	0.5-1.1CM			
FS					
EF	62%				

Comment:

- ⊗ Normal LV end systolic dimensions with normal wall thickness and normal global and regional systolic function.
- ⊗ No SWMA at rest.
- ⊗ Dilated left atrium.
- ⊗ Normal MV thickness and excursion with no mitral regurgitation or significant stenosis.
- ⊗ Mitral inflow shows pseudo-normal E/A ratio.
- ⊗ Aortic valve trileaflet with adequate leaflets thickness without significant stenosis or regurgitation.
- ⊗ Normal RV dimensions with preserved systolic function.
- ⊗ Intact inter-atrial and inter-ventricular septum
- ⊗ Normal appearance of the pericardium. No effusion
- ⊗ No intra-cardiac masses, thrombi or vegetation seen by TTE

Conclusions:

- Normal LV end systolic dimensions with normal global and regional systolic
- Dilated left atrium

Dr. Khaled Alaa, Msc

Patient Name السيدة/ مديحة تكي ابو خزيم
Age / Sex 51 Years / Female
Referred By Herself

Patient ID. 
Request Date 15-Apr-2026 08:17 PM
Printed In 16-Apr-2026 01:32 PM

BIOCHEMISTRY REPORT

Kidney Functions :

1 Serum Creatinine : 0.86 mg/dL

Reference Range :

Normal: 0.6 - 1.1 mg/dL

Doctor's signature:
Enas Mohamed Abdelghafar
Thanks